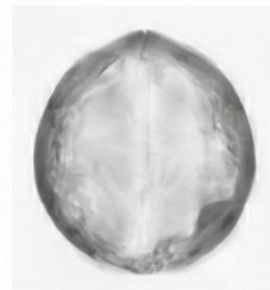
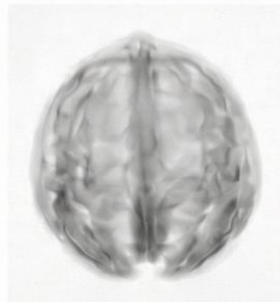
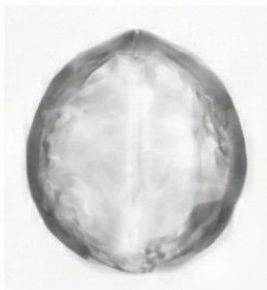


The Health Care Shell Games



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Summary:

An attempt to get initial quantitative measures on the state of Healthcare in America has exposed an underlying **qualitative decline** in healthcare while only the insurance industry and the education establishment benefit financially. Similar to a shell game, where the consumer and the medical profession are actually receiving little to no benefits in return.

#1 Government Shell Games

Healthcare was free then the Government got involved and improved things

National Association of Free and Charitable Clinics

Founded in 2001 under the name of the National Association of Free Clinics, the group changed its name in 2011 to incorporate organizations providing care on a low-cost sliding scale fee model. When founded the organization had ~ 1200 clinics and by 2019 the number was still ~1200. In 2025, that number is ~1400+ but now includes something new “Charitable Pharmacies”, so the number of clinics may still be stagnant.

Patient Dumping

Back in the early 1980s healthcare was often free and the only complaint was long lines. What happened? There were government cuts to community hospitals, an influx of migrants, and doctors refusing to handle patients on government health insurance as the payments were insufficient.

As the funding cuts were taking place, private hospitals began patient dumping the uninsured to publicly subsidized hospitals. This occurred in places like Memphis Tennessee and Cook County Illinois. Why would people pay for healthcare insurance, if the government will pay for their healthcare services? What are government funded hospitals for, if not to cover the uninsured?

This was a government created situation, and the first time the government tried to punt the problem back onto the paying consumers with the EMTALA act.

The Emergency Medical Treatment and Active Labor Act (EMTALA)

- ❖ **Requirement:** If an emergency medical condition exists, the hospital **must stabilize** the patient within its capabilities. They cannot discharge or transfer the patient simply because they cannot pay.
- ❖ **Penalty:** It established stiff civil fines (originally \$50,000 per violation) for hospitals and doctors who negligently "dumped" patients.

- ❖ **Funding:** The Act provided \$0 for patient care.

The end results of EMTALA

- ❖ **Cost Shifting:** To pay for the mandate, hospitals initially raised prices for private insurers (cost-shifting).
- ❖ **The Financial Squeeze:** As managed care (HMOs) grew in the 1990s and refused to pay inflated rates, the "unfunded" nature of EMTALA began to cause the closure of many ERs and trauma centers that could not absorb the costs of uninsured care.
- ❖ **Disproportionate Share Hospital (DSH) Payments:** An already existing government program became the *de facto* funding mechanism for EMTALA. The survival of many inner-city and rural hospitals depends entirely on these payments to offset the costs of the 1986 mandate.

The Affordable Care Act (ACA): The Cost Reduction Plan That Wasn't

The signed ACA law itself was only 906 pages, but the reality is 20,000+ pages of regulations written by federal agencies in support of the ACA. An Everest of regulations will never create market efficiency, but can always add cost. Before we get to the results, below is the actual text regarding cost efficiency provided in the ACA.

This text is found in **Section 1501(a)(2)(F)** of the Patient Protection and Affordable Care Act (Public Law 111–148).

(F) The cost of providing uncompensated care to the uninsured was \$43,000,000,000 in 2008. To pay for this cost, health care providers pass on the cost to private insurers, which pass on the cost to families. This cost-shifting increases family premiums by on average over \$1,000 a year. By significantly reducing the number of the uninsured, the requirement, together with the other provisions of this Act, will lower health insurance premiums.

Section 1501(a)(2)(F) above was never correlated to any actual research or findings. In fact it appears to be simply made up, as future results have revealed.

The Supreme Court Shell Game

This is a summary taken largely from the commentary of constitutional expert Mark Levin

1. **It is a "Finding," not a Law:** It does not *mandate* lower premiums; it simply *states* that they will go down as a justification for the government's authority to enforce the Individual Mandate.
2. **The "Cost-Shifting" Argument:** This text encapsulates the "cost-shifting" theory—that the healthy must be forced to buy insurance to subsidize the sick—which was the central economic and legal justification the Obama administration offered to the Supreme Court.

In the end, the Supreme Court somehow created its own argument in their decision stating the ACA mandate was a tax. The first and only time in history the federal government has forced the American people to buy a product from private industry.

The Tipping Point, a Shell Game with No Reserves

Life, home and auto insurers, have reserve requirements, while healthcare insurers have nothing in reserve for the known coming silver tsunami. The median age in America reached a record high in 2025 at 39.1 years of age. The tipping point is at 40 years of age, below age 40 ~20.5% of lifetime healthcare cost occurs, above age 40 ~79.5% of lifetime healthcare costs occur.

From the key academic study on this topic (Alemayehu & Warner, 2004)

Age Period	% of Lifetime Spending
Childhood (0-19)	~8%
Young Adulthood (20-39)	~12.5%
Middle Age (40-64)	~31%
Senior Years (65-84)	~36.5%
Old Senior Years (85+)	~12%

<https://pmc.ncbi.nlm.nih.gov/articles/PMC1361028/>

US governments have **not mandated any reserve requirements for an aging population.**

By contrast private health insurers in Germany are permitted to accumulate significant funding reserves in the form of "Alterungsrückstellungen" (aging reserves), which help private insurers account for demographic change and medical costs that rise with the age of the insured. Current reserves total around €170 billion. This private pre-funded system is not nearly as vulnerable the US pay-as-you-go public systems.

#2: The Insurance Industry Shell Games

Pharmacy Coverage Game

Why are prescriptions insanely higher through insurance than without? Starting with actual witnessed consumer experience.

Pharmacy: The cost for your medication is \$420 but the good news is your copay is only \$229.

Consumer: How about the cost on GoodRx?

Pharmacy: That would be \$107

Pharmacy: That will be \$570.

Consumer: \$570 dollars? My Doctor said it would be about \$50 dollars.

Pharmacy: I don't know why your Doctor would say that, he is not a Pharmacist?

The most disturbing part of these exchanges is that for some reason some pharmacies are no longer helpful. Initially when quoted a ridiculous price pharmacy staff would often search for a discounted price unprompted. No longer.

This is not anecdotal. Free prescription discount services are saving the insured Billions in prescription cost.

- ❖ GoodRX in 2024 saved consumers on prescription costs approximately 17 Billion, cumulative since 2011 is 85 Billion (32 billion last two years alone, 37 % in the previous two years)
- ❖ SingleCare since its inception in 2014/2015 has saved consumers over \$11 Billion. The \$11 Billion threshold was passed sometime between mid-2023 and mid-2024 .

The reason for over \$96 Billion includes NOT using health insurance for prescriptions or doctor appointments.

Hide the Profits Shell Game

Former Cigna executive Wendell Potter compiled data showing the seven largest publicly traded health insurers (UnitedHealth, CVS/Aetna, Cigna, Elevance, Humana, Centene, and Molina) collectively made \$71.3 billion in profits in 2024, up more than half a billion dollars from 2023. Their CEOs earned a combined \$146.1 million in compensation.

Naturally, the National Association of Insurance Commissioners (NAIC) disputed the numbers. Because the NAIC profit numbers exclude? The profit insurance conglomerates made through their pharmacies and Pharmacy Benefit Managers (PBMs).

Consumers have two bad choices:

- pay far higher than straight out-of-pocket copays for prescriptions and appointments
- or pay the premiums and NOT use their insurance coverage

Why Hide the Profits: The Financial Shift Shell Game

In 2022, there was an abrupt financial shift in the healthcare industry. A report produced by Jakoc Emerson at Becker's Payer Issues exposed a disturbing shift in where healthcare profits occurred.

Third quarter of 2022 profits, payer vs. provider.

Insurance Payers

- UnitedHealth Group: \$5.3 billion, up over 28 percent
- Cigna: \$2.8 billion, up over 70 percent
- Elevance Health: \$1.6 billion, up over 7 percent
- Humana: \$1.2 billion, down 20 percent
- Centene: \$738 million, up over 26 percent
- Molina: \$230 million, up over 60 percent
- CVS Health: \$3.4 billion in losses, attributable to legal settlement

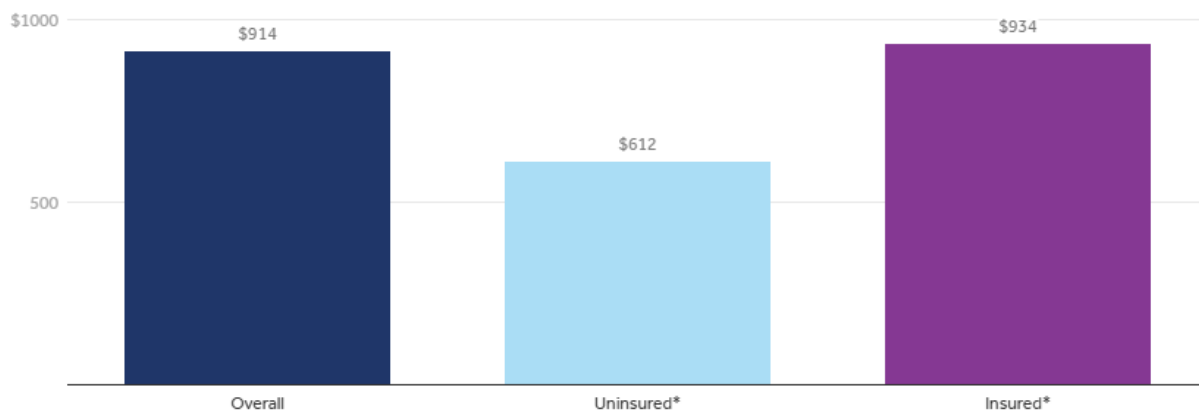
Healthcare Providers

- HCA Healthcare: \$1.13 billion, down over 50 percent
- Universal Health Services: \$182.8 million, down over 16 percent
- Tenet Healthcare \$131 million, down over 70 percent
- Encompass Health Corporation: \$45.4 million, down over 54 percent
- Community Health Systems: \$42 million in losses, down over 137 percent

It does not look good when every provider is on a downward trend but the insurance providers are mainly upward. You have only two payers moving downward, one because of a legal settlement, which also does not look good. The insurance industry is hiding profits, obscuring the shift from providers to payers.

Why the Insured Pay More than the Uninsured?

Average out-of-pocket spending by insurance status, 2021



Note: *There is a statistical difference in the average out-of-pocket spending for insured and uninsured populations.

Source: KFF analysis of 2021 Medical Expenditure Panel Survey data • [Get the data](#) • PNG

Peterson-KFF
Health System Tracker

Why are the insured paying more out of pocket than the uninsured?

- ❖ growth of high deductible healthcare plans (HDHP)
- ❖ copay for doctor visits and prescriptions are higher than paying out-of-pocket
- ❖ many uninsured people simply don't have to pay

The New Don't Pay Method: Freeloaders or Failing System

One in ten adults owe medical debt, with millions owing more than \$10,000. From Jama Networks an analysis of mean Hospital repayment rates ranges from between 42% to 55% full repayment within a year, dependent on different factors. The dataset used includes the uncaptured “patient cost sharing”, where insurance pays a portions of the cost, claims victory and abandons the hospital and patient to deal with the rest of it.

Key Findings:

- “Across all years in our sample, **patients with private or Medicare Advantage insurance paid either 0% or 100% of their owed cost sharing in 92.2% and 94.1% of cases**, respectively” (years 2018 to 2024)
- “our analysis does not include bills that are sold to collections agencies **less than 4% of medical collections included on credit reports are ever reported as paid**”
- “Observed **declines in patient repayment rates coincide with consumer protections removing medical debt from credit reporting**”
(Source: Jama Networks)

The States Jump in: To Hide the Problem

- ❖ Fifteen states have **banned the reporting of medical debt on credit reports**, with nine going into effect in 2025 or January 2026.
- ❖ Now 38% of the population is creating a hidden debt bomb while the insurance industry makes record profits.
- ❖ If medical debt affects the ability of consumers to borrow money, it affects a state’s economy. Forcing additional states to ban reporting of medical debt.

The 15 states with medical debt credit reporting bans:

State	Population (2024)	State	Population (2024)
California	39,431,263	Colorado	5,957,493
New York	19,867,248	Oregon	4,272,371
Illinois	12,710,158	Connecticut	3,675,069
New Jersey	9,500,851	Rhode Island	1,112,308
Virginia	8,811,195	Delaware	1,051,917
Washington	7,958,180	Maine	1,405,012
Maryland	6,263,220	Vermont	648,493
Minnesota	5,793,151	Total	~128,460,000

#3: The Healthcare Quality Shell Games

Attacking the Medical Profession

Keep in mind that all of these **healthcare quality downgrades** have come with **absolutely no savings to the consumer**, but they have increased some profit margins.

The Doctor/PA Sleight of Hand

The medical profession has forcibly replaced Doctors with Physician Assistants (PAs). The standard line being if you want to see a Doctor it will be three weeks but you can see a PA in a day or two. In practicality who can wait three weeks? By then you have likely recovered or checked into an emergency room, so not realistically a choice. Strangely when tested both times the Doctor happened to have a cancellation at just that moment.

Required, Forced Coercion or Outright Bribe

The stated reason for this shell game is a shortage of doctor's in more rural or outlying areas, but in practice, PAs are replacing doctors everywhere? A significant quality downgrade with no cost reduction to the consumer.

From 2010 to 2024 workforce growth:

- ❖ Physicians ~ 27%
- ❖ Physicians Assistants ~ 127%

PAs are highly profitable, with lower cost and reimbursement at 85% of the physician fee schedule. Since PAs require a doctor present, the doctor can finally stop fighting the insurance company for reimbursement and cash in by becoming an office manager. Significant quality downgrade, with increased insurance profits, and nothing for the consumer.

PA Shell Game: Six to Nine Years Qualified, but only Two or Three Count

There are two significant requirements to apply for state licensure. You must pass the Physician Assistant National Certifying Exam (PANCE), administered by the National Commission on Certification of Physician Assistants (NCCPA).

Strange how the doctors have ultimate legal liability for the PAs with no formal authority or approval rights over the PANCE exam.

The second requirement is you must graduate from a PA program accredited by the Accreditation Review Commission on Education for the Physician Assistant (ARC-PA).

The University Shell Game, Expanded to Who's Needs?

We have an education system with shrinking demand that has generated \$1.81 Trillion in outstanding student loan debt, providing degrees that often did not pay for themselves now in charge of **cashing in** and recycling those degrees into PAs. While competing for the same pool of useful and useless degrees, in what appears to be a “race to the bottom” What could go wrong?

Researching the five most populous states (California, New York, Florida, Texas, and Pennsylvania) **revealed a loose set of qualifications broadly expanded to create as many pre-qualified candidates as possible.**

Watering Down the Requirements

- ❖ You need any four-year degree to get into a PA program and it will take two to three years to complete your education. Then you can write the PANCE exam to become a licensed PA.
- ❖ Programs that require **zero medical experience or education** are available in every state but California. Across the five states, there were 51 occupations with accepted medical experience at 500 to 0 hours so not really a requirement. Most institutions still require some medical experience but all states accept volunteer hours, and some accept shadowing hours.
- ❖ Aptitude testing (GRE or MCAT) is **almost never required**

CASPer: More focus on image than aptitude?

Computer-Based Assessment for Sampling Personal Characteristics (CASPer), is an online situational judgment to evaluate soft skills like empathy, ethics, and problem-solving, rather than academic knowledge (i.e. Bedside Manner) and is commonly required.

GRE and MCAT testing which heavily rely on cognitive skills like critical thinking, problem solving, and reasoning, measuring general aptitude for advanced study are rarely required.

Goodbye Doctors?

Does this **sound like the 85% of a doctor consumers are paying for?** They are not. Which makes the next move transparent. The justification to reduce Doctor\PA pay is already in place. More research is required to determine if we are at the tipping point yet.

The Private Equity Shell Game: Marginalizing the Medical Professionals

Private Equity has been buying smaller and rural hospitals, now owning 24% of hospitals but only 16% of hospital beds.

The story vs. reality

- ❖ Often marketed as an effort to shore up struggling hospitals by providing needed capital investment
- ❖ More often **private equity firms buy financially healthy hospitals so they can take on more debt while still generating revenue**

The **bottom line result** is making more employees part time, reducing staff hours and benefits, reducing the quality of care and more people dying.

“private equity hospitals reduced salary expenditures in the emergency department by 18 percent and in the ICU by 16 percent compared to hospitals not acquired by private equity. They also reduced hospital-wide full-time employees by an average of 11.6 percent and salary expenditures by 16.6 percent relative to non-private equity hospitals.” (source Harvard Medical School)

“Medicare beneficiaries in the emergency departments of private equity hospitals experienced seven additional deaths per 10,000 visits after acquisition relative to hospitals that were not acquired by private equity, the team found. This rise represented a 13 percent increase from a baseline of 52 deaths per 10,000 visits.” (source Harvard Medical School)

The Extremely Suspicious Attack on Nurses

For very questionable and nonsensical logic, the Department of Education removed the designation of Nursing as a professional degree.

“Department of Education data indicates that **95% of nursing students borrow below the annual loan limit and therefore are not affected by the new caps.**

Further, placing a cap on loans will push the remaining graduate nursing programs to reduce their program costs, ensuring that nurses will not be saddled with unmanageable student loan debt.

It is important to remember that **the loan limits are limited to graduate programs and have no impact on undergraduate nursing programs**, including four-year bachelor’s of science in nursing degrees and two-year associate’s degrees in nursing. “80% of the nursing workforce does not have a graduate degree.” (Source: Department of Education)

Reviewing the Logic

The middle paragraph is nonsensical and a repeat of the ACA. We are going to take away the money to keep down the cost of education. Removing the professional designation from nurses for NOT running up enough student loan debt. It would make considerably more sense to cut off the education that is not in demand, while nursing remains in high demand.

The first and third paragraph tell the real story, **Nurses are a threat to the Doctor\PA shell game**. The university system and the insurance industry are protecting their cash cow**—PAs.

Taking away the professional designation pulls the financing from Nurses and puts them down into the same Department of Education category as PAs!

As of 2025, from the BLS the Median annual wage for PAs is \$133,260, which is equivalent to the top 10% of RNs (Registered Nurses with a four-year degree) at \$135,320. RNs Median annual wage is only \$93,600. (Source: Bureau of Labor Statistics)

You have an army of much more qualified candidates making far less money. You have Nurses with four-year degrees and potentially decades of experience vs. PAs with totally unrelated degrees and no experience other than what appears to be 1.5 years of rotation watching Doctors work with patients. The Department of Education is financially cutting off nurses, from replacing the less qualified PAs. A move that only benefits the university system, the insurance industry and not the public good.

A Look at Solutions

Start the End Game as Soon as Possible

Whoever tells the story controls the story

The AMA has a monopoly on the language of American healthcare billing. Federal law (HIPAA) mandates the use of CPT codes for billing physician services. This unique arrangement—a private copyright on a federally mandated standard—is why the database is not public domain.

Bring in Motivated Partners

The **medical offices** that have fought the Deny, Delay, Defend method for decades. Needing complex software to tell them if the insurance providers are paying them more or less than last year.

The **hospital networks** that are continually suffering lower margins and losses as the insurance industry continually makes more and more money.

These are the partners with the information to expose insurance industry shenanigans. Ties the CPT codes already there into all the denials, delays, regional monopolies, payment deltas between regions, rural vs metropolitan, Doctor visits vs PA visits etc.

Protect the Consumers and End the Shell Games

Without getting into the analytics and technology details, compiling this data will easily expose all the shell games, shift income back from the insurers to the providers, and reduce insurance margins to reasonable levels. Ending a shell game that has spanned decades.

What Should and Shouldn't Happen

- ❖ Do NOT let the insurance industry have any input or control over data collection methodology
- ❖ Do NOT let Washington do it by putting out a bid process, they could take years to complete a simple task, for something with limited startup cost
- ❖ Do NOT let the industry play the HIPAA card (Health Insurance Portability and Accountability Act), there is a very quick solution and the insurance industry can learn about it after the fact
- ❖ Do bring in and work with medical office and hospital software vendors if it could expedite the process (unlikely to be needed)
- ❖ Do NOT quit the process out of fear, the process will create the information medical professionals require at minimal cost.

The Nursing Shell Game: Time to Defend the Profession

Reasons why RNs need to stay above PAs

- ❖ They have considerably more qualifications, education and experience and there is no legitimate justification for paying PAs more than RNs
- ❖ The argument that PAs are needed to address a doctor shortage is completely inaccurate if PAs are not limited to rural and outlying areas
- ❖ Healthcare insurers and the educational establishment are profiting from the current PA arrangement that significantly downgrades the quality of Healthcare by bypassing the often more qualified nursing profession
- ❖ It would take far less cost and provide far better qualified healthcare to convert RNs to PAs in metropolitan areas where there is no shortage
- ❖ PAs are undoubtedly not qualified to replace Doctor's in specialist niches, RNs if they have the relevant work experience can clearly be more qualified

- ❖ It would be a considerable upgrade and more efficient to convert the more qualified RNs to become PAs. RNs could be converted to PAs in less time be more qualified and be replaced by other types of nurses faster and it would cost the healthcare industry considerable less expense
- ❖ Much higher entry qualifications requiring SAT/ACT test scores and later a TEAS exam (no academic testing for PAs). Required GPAs of 3.6 to 4.0 as opposed to only 3.0 for PAs.

The nursing shortage is regional and very specific not universal across the country. In the Salt Lake Metropolitan area, the medical establishment is requiring four-year degrees to maintain employment in nursing and nurses are flying to Nevada on a weekly rotating basis just to get work. Clearly not a nursing shortage, not a legitimate need for PAs, and abusive to the nursing profession.

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